

DISCHARGE SUMMARY FOR OMAR HADDAD, MRN 008-33009

Omar Haddad, a 77-year-old male, was admitted on February 28, 2026, and discharged on March 3, 2026.

DIAGNOSTIC SUMMARY

The patient was diagnosed with community-acquired pneumonia and chronic obstructive pulmonary disease (COPD).

IMAGING STUDIES

A chest X-ray revealed a right lower lobe infiltrate.

MEDICATION REGIMEN

The patient was prescribed azithromycin 500 mg orally on day one, followed by 250 mg daily for days two through five. Additionally, prednisone 40 mg orally was administered daily for five days. The patient was also instructed to use a tiotropium inhaler, inhaling two puffs twice daily.

VITAL SIGN OBSERVATIONS

The patient's oxygen saturation was 91% on room air, and respiratory rate was 24 breaths per minute.

THERAPEUTIC INTERVENTIONS

Supplemental oxygen therapy was provided as needed.

DISCHARGE INSTRUCTIONS

The patient was advised to complete the full course of antibiotics and schedule a follow-up appointment with the pulmonary specialist in two weeks.

Discharge Summary for Omar Haddad, MRN 008-33009, born 1948-08-19.

Admitted on 2026-02-28 and discharged on 2026-03-03.

Diagnoses included community-acquired pneumonia and chronic obstructive pulmonary disease (COPD).

Imaging studies revealed a right lower lobe infiltrate on chest X-ray.

Medications prescribed were azithromycin 500 mg orally on day one, followed by 250 mg daily for days two through five; prednisone 40 mg orally daily for five days; and tiotropium inhaler, administered with two puffs inhaled daily.

Vital signs observed were a SpO2 of 91% on room air and a respiratory rate of 24 breaths per minute.

Supplemental oxygen therapy was administered as a procedure.

The plan is to complete the course of antibiotics and schedule a pulmonary follow-up appointment in two weeks.

Discharge Summary for Omar Haddad, MRN 008-33009, born 1948-08-19.

Diagnoses upon discharge included community-acquired pneumonia and chronic obstructive pulmonary disease (COPD).

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During the hospital stay, the patient received supplemental oxygen therapy.

The plan for follow-up care included completion of the prescribed antibiotic course and a pulmonary follow-up appointment in two weeks.

Here is a rewritten version of the clinical note:

DISCHARGE SUMMARY

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